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Name	: **Mr. Manak Sahu**	Age / Sex	: 36 Year(s) / Male	Collected On	: 02/05/2024 08:33 AM
Patient ID	: OHP9MW6G785661	Ref. Doctor	:	Received On	: 02/05/2024 10:33 AM
Visit No.	: BL81823501			Reported On	: 02/05/2024 12:56 PM

BL81823501

Test	Results	Units	Biological Reference Interval
BIOCHEMISTRY			
Blood Glucose F- fluoride plasma,GOD-POD	99	mg/dL	Random: 70-140 Fasting: 70-99 Post Prandial: 70-140

Blood Sugar also known as Blood glucose test is commonly performed to diagnose and monitor Diabetes Mellitus. In common practice the blood sugar is measured and reported as 3 different tests based on the meal intake status of an individual.

Low blood glucose level may be due to Overdose Insulin, Insulinomas, Starvation, Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease,Hypopituitarism, Hypothyroidism, Severe infections.

High blood glucose level most frequently indicate diabetes, but many other diseases and conditions can also cause elevated blood glucose.e.g.Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance),Cushing syndrome, Hyperthyroidism, Pancreatic cancer, Pancreatitis.

The reference interval has been referred from the American diabetes Association (https://www.diabetes.org/a1c/diagnosis). Kindly consider the reference range of blood sugar according to the fasting status or post meal status.

Name of Test	Meal Status	Reference Range
Fasting Blood Sugar	Overnight Fasting	70-99
Random Blood Sugar	Sample taken irrespective of meal status	70-140
Post Prandial Sugar	Sample taken after 2 Hrs of meal consumption	70-140

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Test	Results	Units	Biological Reference Interval
LFT - LIVER FUNCTION TEST Serum			
Total Bilirubin DSA	1.15	mg/dL	Adult: 0.2-1.3 Neonate: 1.0-10.5
Direct Bilirubin Calculated	0.22	mg/dL	Neonate: 0.0-0.6 Adult: 0.0-0.3
Indirect Bilirubin Dual wavelength	0.93	mg/dL	Adult: 0.1-1.1 Neonate: 0.6-10.5
Aspartate Transaminase(AST/ SGOT) MDH, UV Kinetic	36	U/L	17-49
Alanine Transaminase(ALT/ SGPT) LDH, UV kinetic	46	U/L	< 50
Alkaline Phosphatase PNPP, AMP Buffer	62	U/L	38-126
Gamma-Glutamyl Transferase (GGT) SZAZ Carboxylated Substrate	16	U/L	15-73
Total Protein Biuret	7.8	g/dL	6.0-8.3
Albumin Bromo-Cresol Green	4.7	g/dL	3.5-5.0
Globulin Calculated	3.2	g/dL	2.3-3.5
A/G ratio Calculated	1.5		0.8-2.0

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Test	Results	Units	Biological Reference Interval
SGOT/SGPT- RATIO Calculated	0.8		0.7-1.4

Total bilirubin is invariably increased in jaundice. Causes of jaundice are prehepatic, resulting from various hemolytic diseases; hepatic, resulting from hepatocellular injury or obstruction; and posthepatic, resulting from obstruction of the hepatic or common bile ducts.
Aspartate aminotransferase is present in high activity in heart, skeletal muscle, and liver. Increased serum AST activity commonly follows myocardial infarction, pulmonary emboli, skeletal muscle trauma, alcoholic cirrhosis, viral hepatitis, and drug-induced hepatitis.
Alanine aminotransferase is present in high activity in liver, skeletal muscle, heart, and kidney. Serum ALT increases rapidly in liver cell necrosis, hepatitis, hepatic cirrhosis, liver tumours, obstructive jaundice, Reye's syndrome, extensive trauma to skeletal muscle, myositis, myocarditis, and myocardial infarction.
Alkaline phosphatase is elevated in fever and increased bone metabolism, for example, in adolescents and during the healing of a fracture; primary and secondary hyperparathyroidism; Paget's disease of bone; carcinoma metastatic to bone; osteogenic sarcoma; and Hodgkin's disease if bones are invaded. Hepatobiliary diseases involving cholestasis, inflammation, or cirrhosis increased ALP activity; also increased in renal infarction and failure and in the complications of pregnancy. Low ALP activity may occasionally be seen in hypothyroidism.
Serum GGT is a sensitive indicator of hepatobiliary disease and is useful in the diagnosis of obstructive jaundice and chronic alcoholic liver disease, in the follow-up of chronic alcoholics undergoing treatment, and in the detection of hepatotoxicity. GGT is more responsive to biliary obstruction than AST, ALT, or ALP.
Total serum protein levels can be used for evaluation of nutritional status. Causes of high total serum protein concentration include dehydration, Waldenström's macroglobulinemia, multiple myeloma, hyperglobulinemia, granulomatous diseases, and some tropical diseases. Causes of low total serum protein concentration include pregnancy, excessive intravenous fluid administration, cirrhosis or other liver diseases, chronic alcoholism, heart failure, nephrotic syndrome, glomerulonephritis, neoplasia, protein-losing enteropathies, malabsorption, and severe malnutrition.

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Test	Results	Units	Biological Reference Interval
Glycosylated Haemoglobin - (HbA1c)			
Whole Blood			
Glycosylated Haemoglobin (HbA1c) HPLC	4.7	%	Normal: < 5.7 Pre-Diabetes: 5.7-6.4 Diabetes: >= 6.5
Mean Blood Glucose Calculated	88	mg/dL	

HbA1C is used to monitor fluctuations in blood glucose concentration in the past 8 to 12 week's period.
The reference interval defined as per American Diabetes Association guidelines 2016:

a) Less than 5.7% : Non Diabetic
b) 5.7 to 6.4% : at increased risk of developing diabetes in the future
c) More than 6.5% : Diabetic
d) Therapeutic glycemic target:
i. Adults: less than 7%
ii. Children with Type 1 diabetes : less than 7 %
e) Pregnant diabetic patients : less than 6.5%

Note:
• Targets may be individualized based on: Age/life expectancy, Comorbid conditions, Diabetes duration , Hypoglycemia status, Individual patient considerations
Reference: American Diabetes Association. Standards of medical care in diabetes—2021.
Mean Blood Glucose is average Blood glucose which directly correlates with A1C, reported in the same units as blood sugar levels (mg/dl). Thus it reflects the average glucose concentration in the past 8 to 12 weeks period. This should not be compared with Fasting or Post prandial or random blood sugar which measures glucose concentration at that point of time of testing.

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Test	Results	Units	Biological Reference Interval
Kidney Function Test Serum			
Urea Urease	20	mg/dL	19-43 Newborn: 6.5-25.5
Creatinine Enzymatic	0.9	mg/dL	0.66-1.25
Blood Urea Nitrogen (BUN) Calculated	9.3	mg/dL	6-20
BUN Creatinine Ratio Calculated	10.33	Ratio	10-20
Uric Acid Uricase	6.8	mg/dL	3.5-8.5
Calcium Arsenazo Method	9.2	mg/dL	8.4-10.2
Phosphorus Phosphomolybdate formation	3.5	mg/dL	2.5-4.5
eGFR CKD EPI	114	ml/min/1.73 sq m	Normal: >= 90 Mild decrease: 60-89 Mild moderate decrease: 45-59 Severe decrease: 15-29 End stage kidney disease: < 15

ELECTROLYTES
Direct ISE

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Test	Results	Units	Biological Reference Interval
Sodium	140	mmol/L	137-145
Potassium	4.3	mmol/L	3.5-5.5
Chloride	102	mmol/L	98-107

Electrolytes are used to find and monitor conditions that affect the balance of fluids, electrolytes and acidity in the body and also helps nerves and muscles work properly.
High **urea** and **BUN** levels is suggestive of poor kidney function due to acute or chronic kidney diseases, decreased blood flow to the kidneys as in congestive heart failure, shock, stress, recent heart attack or severe burns; bleeding from the gastrointestinal tract; conditions that cause obstruction of urine flow; or dehydration.Low urea levels are not common and are not usually a cause for concern. They can be seen in severe liver disease or malnutrition but are not used to diagnose or monitor these conditions. Low urea levels are also seen in normal pregnancy.
Creatinine is elevated in Kidney disease, damage, infection, urinary tract obstruction, reduced blood flow to the kidneys in case of shock, congestive heart failure, complication of diabetes.
High levels of **uric acid** are seen in kidney disease,pre-eclampsia, purine rich food, alcoholism, and side effects of cancer treatment.
Low levels of **calcium** may be due to hypoparathyroidism, kidney failure, pancreatitis, malnutrition, disorder in calcium absorption. High levels of calcium may be due to hyperparathyroidism, hyperthyroidism, sarcoidosis, drugs like diuretics, and excessive calcium supplementation.
Causes of high serum **phosphorus** include dehydration, hypoparathyroidism, hypervitaminosis D, metastases to bone, sarcoidosis, pulmonary embolism, renal failure, and diabetes mellitus with ketosis. Low serum phosphorus is found in primary hyperparathyroidism and other causes of serum calcium elevation, sepsis, vitamin D deficiency, renal tubular disorders, chronic hemodialysis, vomiting, and occasionally with decreased dietary phosphate intake.

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Test	Results	Units	Biological Reference Interval
Lipid Profile with calculated LDL			
Serum			
Total Cholesterol CHOD-POD	110	mg/dl	< 200
Triglycerides GPO-POD	**183**	mg/dL	< 150
HDL Cholesterol Direct	**32**	mg/dL	> 50
Non-HDL Cholesterol Calculated	78	mg/dL	< 130
LDL Cholesterol Calculated	41	mg/dL	< 100
VLDL Cholesterol Calculated	**37**	mg/dL	< 30
Cholesterol / HDL Ratio Calculated	3.4		3.30-4.40
LDL:HDL RATIO Calculated	1.3		0.5-3.0
HDL/LDL RATIO Calculated	0.8	Ratio	> 0.4

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Test	Results	Units	Biological Reference Interval

REMARKS	TOTAL CHOLESTEROL(mg/dL)	TRIGLYCERIDE(mg/dL)	LDL CHOLESTEROL(mg/dL)
Optimal	<200	<150	<100
Above Optimal	-	-	100-129
Borderline	200-239	150-199	130-159
High	>=240	200-499	160-189
Very High	-	>=500	>=190

Lipid profile is a group test consisting of various lipids. Lipid profiles are generally collected with overnight fasting. **However, Recent guidelines have recommended non fasting samples for lipid profile for assessment of cardiovascular risk. The details for the study can be checked at** https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2733560

In certain instances measurements in the same patient can show physiological and analytical variations. In such cases three serial samples at an interval of 1 week each are recommended for Total cholesterol, TG, HDL and LDL.

Cholesterol levels are increased in primary hypercholesterolemia; secondary hyperlipoproteinemia, including nephrotic syndrome; primary biliary cirrhosis; hypothyroidism; and in some cases, diabetes mellitus. Low cholesterol levels may be found in malnutrition, malabsorption, advanced malignancy, and hyperthyroidism.

Triglyceride levels are used in the diagnosis and treatment of patients with diabetes mellitus, nephrosis, liver obstruction, other diseases involving lipid metabolism, or various endocrine disorders.

High Density Lipoprotein (HDL) cholesterol levels is used to evaluate the risk of developing coronary heart disease (CHD). The risk of CHD increases with lower HDL cholesterol concentrations.

LDL (low-density lipoprotein) cholesterol level, sometimes called “bad” cholesterol, makes up most of our body’s cholesterol. High levels of LDL cholesterol raise your risk for heart disease and stroke.

Very-low-density lipoprotein (VLDL) cholesterol is produced in the liver and released into the bloodstream to supply body tissues with triglycerides. High levels of VLDL cholesterol have

been associated with the development of plaque deposits on artery walls, which narrow the passage and restrict blood flow.

Dr. Sanchit Singhal
MD Pathologist

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Test	Results	Units	Biological Reference Interval
IMMUNOLOGY			
Vitamin B12 Serum,CLIA	**<159**	pg/mL	239-931
Vitamin D (25 OH Cholecalciferol) Serum,CLIA	49.4	ng/mL	Deficient: < 20 Insufficient: 20-30 Sufficient: 30-100 Toxicity: > 100

For the diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25 (OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.
During the monitoring of oral vitamin D therapy - suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 months of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal (especially winter) & individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism.

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Test	Results	Units	Biological Reference Interval
Thyroid function test (TFT) Serum,CLIA			
Total T3	1.31	ng/mL	0.97-1.69
Total T4	7.40	ug/dl	5.53-11.0
Thyroid Stimulating Hormone (TSH)	**4.998**	µIU/mL	0.400-4.049

Comment Kindly correlate clinically and refer to the interpretation provided below.

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Test	Results	Units	Biological Reference Interval

Thyroid Stimulating Hormone (TSH), also called Thyrotropin is a hormone secreted into the blood by the Pituitary gland (a gland present in the brain)). It signals the thyroid gland to make and release the thyroid hormones (T3 & T4) into the blood.

High TSH level indicates that the thyroid gland is not making enough thyroid hormone (primary hypothyroidism).

Low TSH level usually indicates that the thyroid is producing too much thyroid hormone (hyperthyroidism).

Factors influencing TSH levels:

Fasting: TSH level shows a significant decline after meal intake in comparison to fasting values. This may have clinical implications in the diagnosis and management of hypothyroidism, especially Subclinical hypothyroidism.1

Circadian Rhythm: TSH levels follow a circadian variation, reaching peak levels between Morning 2 – 4 am and at a minimum between Evening 6-10 pm. The variation is of the order of 50%. hence time of sample collection during a day can significantly influence on the measured serum TSH concentrations.2

Other Factors: Genetics, Poisonous substances and radiation exposure, Inflammation of the thyroid gland, Deficiency or excess of iodine in the diet, Pregnancy, Certain medications – antidepressants, cholesterol lowering drugs, chemotherapy drugs, steroids, Thyroid cancer.

References:

1. Indian Journal of Endocrinology and Metabolism 18(5):p 705-707, Sep–Oct 2014.)

2. http://www.pnei-it.com/1/upload/thyrotropin_secretion_patterns_in_health_and_disease.pdf

In pregnant females the reference range of TSH differs. Please refer the table below for the same:-

PREGNANCY	TSH REFERENCE RANGE (uIU/ml)
1st Trimester	0.100-2.500
2nd Trimester	0.200-3.00
3rd Trimester	0.300-3.00

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Test	Results	Units	Biological Reference Interval
HEMATOLOGY			
Complete Blood Count Whole Blood			
RBC count DC Impedance method	4.71	mill/cu.mm	4.5-5.5
Haemoglobin (Hb) Cyanide-free SLS method	**11.4**	gm/dL	13.0-17.0
Haematocrit(PCV) Calculated	**36.9**	%	40-50
Mean Corpuscular Volume (MCV) Based on RBC Histogram	**78.4**	fL	83-101
Mean Corpuscular Haemoglobin(MCH) Calculated	**24.1**	pg	27-32
Mean Corpuscular Haemoglobin Concentration (MCHC) Calculated	**30.9**	g/dL	31.5-34.5
Red cell distribution width (RDW) Calculated	**16.3**	%	11.6-14.0
Mentzer Index Calculated	16.6	Index	Beta Thalassemia trait: < 14 Iron deficiency anaemia: >= 14
Sehgal index Calculated	1305.0	Index	Beta Thalassemia trait: < 972 Iron deficiency anaemia: >= 972

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Total WBC count - TC DC Impedance method	6430	cells/cu.mm	4000-10000
Differential Leucocyte Count - DC			
Neutrophils Flow Cytometry	58.8	%	40-80
Lymphocytes Flow Cytometry	25.4	%	20-40
Monocytes Flow Cytometry	6.3	%	2-10
Eosinophils Flow Cytometry	**8.4**	%	1-6
Basophils Flow Cytometry	1.1	%	0-2
Absolute Neutrophil Count Calculated	3781	/ cu.mm	2000-7000
Absolute Lymphocyte Count Calculated	1633	/ cu.mm	1000-3000
Absolute Monocyte Count Calculated	405	/ cu.mm	200-1000
Absolute Eosinophil Count Calculated	540	/ cu.mm	20-500
Absolute Basophil Count Calculated	71	/ cu.mm	0-100
Neutrophil Lymphocyte Ratio (NLR) Calculated	2.3	Ratio	1.0-3.0

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Test	Results	Units	Biological Reference Interval
Platelet count DC Impedance method	196	10^3/ul	150-450
Platelet hematocrit Calculated	0.306	%	0.20-0.50
Mean Platelet Volume (MPV) Calculated	**15.7**	fL	7-13

1. Reference Ranges are in accordance with Dacie & Lewis Practical Hematology International Edition (12th).
2. As per International Council for Standardization in Hematology's recommendations Differential Leucocyte counts are additionally reported in Absolute numbers in each cell per unit volume of blood.

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Dr. Abhijay Dharmadhikari
DPB

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Test	Results	Units	Biological Reference Interval
CLINICAL PATHOLOGY			
Urine Complete Analysis Urine			
PHYSICAL EXAMINATION Manual			
Volume	35	ml	
Colour	Pale yellow		Pale yellow
Appearance	Clear		Clear
CHEMICAL EXAMINATION			
pH Double indicator method	6.0		5.0-8.0
Specific gravity Refractive Index	1.005		1.001-1.035
Urine Protein Protein error of pH indicator	Nil		Nil
Glucose Enzyme method GOD POD	Nil		Nil
Ketone bodies Dipstick	Nil		Nil
Bilirubin Azo coupling method	Nil		Nil

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Blood Peroxidase activity	Negative		Negative
Urobilinogen Azo coupling method	Normal		Normal
Leucocyte Esterase Granulocyte esterase method	Negative		Negative
Nitrites Griess method	Negative		Negative
MICROSCOPY EXAMINATION Flow digital Imaging			
Pus cells	1-2	/hpf	0-5
Epithelial cells	0-1	/hpf	0-2
RBCs	Nil	/hpf	0-2
Granular Casts	Nil	/hpf	Nil
Hyaline Casts	Nil	/hpf	Occasional
Uric acid Crystals	Nil	/hpf	Nil
Phosphate Crystals	Nil	/hpf	Nil
Calcium Oxalate Crystals	Nil	/hpf	Nil

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Amorphous urates	Nil	/hpf	Nil
Amorphous phosphates	Nil	/hpf	Nil
Yeast	Nil	/hpf	Nil
Bacteria	Nil	/hpf	Nil
Parasites	Nil	/hpf	Nil
Mucus	Nil		Nil

Increased **protein** in urine is seen in dehydration, kidney disorders, heart failure and transplant rejection. 24 hour urine protein and Protein/creatinine ratio in a random urine sample recommended if increased.
Glucosuria can be seen in kidney disorders, uncontrolled diabetes mellitus, hormonal disorders, and pregnancy. To be correlated with plasma glucose levels.
Ketonuria is seen in physical exercise, starvation, severe vomiting, exposure to cold, uncontrolled diabetes (diabetic ketoacidosis)
Increased **bilirubin** levels should be followed up with Liver function tests and indicates conjugated hyperbilirubinemia.
Increased **urobilinogen** can be seen due to haemolysis, megaloblastic anaemia and haemorrhage in tissues. Urobilinogen is absent or reduced in obstructive liver disease and antibiotic therapy.
RBCs in urine (Haematuria) can be seen in anticoagulant therapy, bleeding diathesis and traumatic catheterization history to be looked into. Dysmorphic RBCs suggestive of glomerular pathology. Non glomerular diseases line calculus, infections, tumours, after strenuous exercise and diseases of the prostate.
Increase in **pus cells** are elevated in cases of UTI, to be correlated with urine culture, if clinically indicated. Infection can be in either the upper or lower urinary tract or with acute glomerulonephritis, tubule interstitial nephritis. Leucocyte esterase detects esterase enzyme released from the granules of leucocytes.
Infected urine may contain considerable amounts of **nitrite** as a result of bacterial nitrate reductase activity, and detection of nitrite in urine is routinely used in the diagnosis of bacterial cystitis. It is indicative of the requirement of Urine culture and sensitivity testing for identification and treatment of UTI.
Hyaline casts are seen normally (not associated with disease states); seen after strenuous exercise and with non renal diseases, such as dehydration.
Granular casts can be seen in acute glomerulonephritis and pyelonephritis.

Dr. Sanchit Singhal
MD Pathologist

Dr. Abhijay Dharmadhikari
DPB

Name	: **Mr. Manak Sahu**	Age / Sex	: 36 Year(s) / Male	Collected On	: 02/05/2024 08:33 AM
Patient ID	: OHP9MW6G785661	Ref. Doctor	:	Received On	: 02/05/2024 10:33 AM
Visit No.	: BL81823501			Reported On	: 02/05/2024 12:56 PM

BL81823501

Test	Results	Units	Biological Reference Interval
SEROLOGY			
Anti-HCV antibodies Serum,CLIA	0.03	S/C Units	Non-reactive: < 0.90 Reactive: >= 1.00 Borderline: 0.90-1.00

- The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with HCV. HCV antibodies may be undetectable in some stages of the infection and in some clinical conditions.

VDRL Serum,Immunochromatography	Non-reactive	Non-reactive

VDRL is a non-treponemal serological test.
Biological false positive reaction can occur in:

- Viral infections
- Malaria
- Following vaccination
- Autoimmune disease such as SLE and RA
- Leprosy

Syphilis can be excluded in patients with biological false positive reactions by doing a treponemal(specific) serological test such as TPHA or FTA-Abs.

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BL81823501

Test	Results	Units	Biological Reference Interval
HIV 1 & 2 Serum,CLIA	0.08	S/C Units	Non-reactive: < 0.90 Reactive: >= 1.00

- The VITROS HIV Combo test uses 3 recombinant antigens derived from HIV–1 envelope (env 13), HIV-1 group O envelope (env 70-3) and HIV–2 envelope (env 31). These antigens detect antibodies to HIV–1 and antibodies to HIV–2 in the same test.
- The VITROS HIV Combo test also uses antibodies to HIV p24 antigen to enable detection of HIV p24 antigen that may be present in early seroconversion before the onset of antibody response enabling earlier diagnosis of HIV infection.
- The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. Repeat testing after 3 to 6 months later is advised if clinically indicated.

Test	Results	Units	Biological Reference Interval
HBsAg Serum,CLIA	0.13	S/C Units	Non-reactive: < 0.90 Reactive: >= 1.00 Borderline: 0.90-1.00

- The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus. Levels of HBsAg may be undetectable both in early infection and late after infection.

-- End of Report --

MC-6367

Tests marked with NABL symbol are accredited by NABL vide Certificate no MC-6367

Dr. Sanchit Singhal
MD Pathologist

Dr. Alekya Seerapu
MD Pathologist

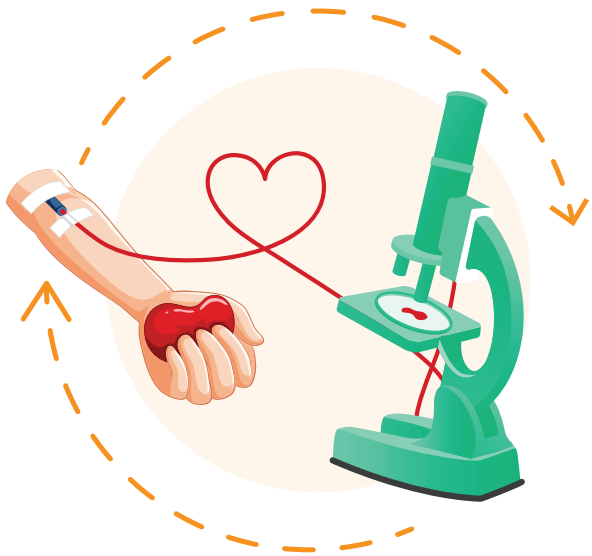
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IN MATTERS RELATED TO HEALTH, FASTER IS BETTER.

We equate showing care to showing urgency.

Blood starts deteriorating the minute it leaves the human vein, unless stored right. We have built our logistics to achieve the **fastest vein-to-machine testing time** in the industry, and in our journey ahead will strive to reduce it further as much as possible.





HOW ARE WE FASTER THAN OTHERS?

We identified two gaps in diagnostics: **sample transportation time** and **processing delays.**

We have solved for these problems by our approach of:

NO COLLECTION CENTRES

Your samples are sent straight to our labs at the right temperature.

NO BATCH TESTING

We don't batch your samples to save costs. They are tested the moment they come in.

NO TIME WASTED

Since we test your blood the fastest, you get the most accurate reports in the industry.

Medically designed checkups only at Orange Health Labs
Allergy Checkups | Hormonal Health | Cancer Screening | Heart Health Markers